

The screening exists. Completion is what's missing.

A partnership brief on closing the gap between preventive care that is offered and preventive care that is actually completed — across Bangladesh's community-clinic screening system.

Bangladesh's community-clinic screening program has national reach and a registry — yet reports low uptake and loss to follow-up. That is a completion-and-fidelity gap, not a science gap. The Prevention Adoption Initiative is a disease-agnostic engine that finds who is overdue, removes friction, and drives completion — with an explainable dashboard. I am looking for a first partner to pilot it and scale it.

THE PROBLEM — ADOPTION, NOT SCIENCE

69%

of total health spending is paid out-of-pocket (WHO, 2025) — a single hospitalization can push a household into poverty.

<1% of GDP

public health spending (World Bank, 2021) — prevention is under-resourced where it protects the economy most.

Reach, low uptake

the national cervical & breast screening program has community-clinic reach and a registry, but its own evaluation reports low uptake and loss to follow-up.

THE APPROACH — THREE PILLARS, ONE COMPLETION LOOP

Service redesign & friction reduction

Make the completed path the easiest one — defaults, mailed at-home tests, one-click scheduling, barrier removal.

Behaviorally-configured data infrastructure

Registries + dashboards that surface who is overdue and where patients drop out; automated reminders; escalation.

Implementation enablement

Training so a site can run and sustain the toolkit itself — the pillar the evidence says matters most.

Identify → **Reduce friction** → **Communicate** (literacy-tailored) → **Escalate** (community-health-worker outreach) → **Measure**, incl. diagnostic follow-up after a positive result.

THE DEMONSTRATION

Embed the completion loop into a program that already exists.

Rather than build something new, the engine embeds into the existing ~14,000 community clinics and the national electronic registry — piloted in a few upazilas, aligned to the national NCD and screening strategy — to close the uptake and follow-up gaps the program's own evaluation reports. Disease-agnostic by design: the same loop applies to cervical and breast screening, hypertension, diabetes follow-up, and immunizations.

THE ASK

Different partners, different value — the same completion engine.

Private healthcare

Completion is revenue — more screenings mean more diagnostics, higher retention, and stronger payer & corporate-wellness contracts.

Government

Strengthen a system that exists — DGHS NCDC and the Community Clinic Health Support Trust, piloted in a few upazilas.

Research & NGO

icddr,b, BRAC, Marie Stopes — turn a pilot into publishable, scalable evidence and open donor funding.

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An HSREP initiative in development. All designs are illustrative and future-tense;
no patient data has been collected. Operational materials will be reviewed by
counsel before any launch.

